

We help elective clinics improve the operational systems that determine whether patient enquiries become booked treatments.

PREPARED FOR

SAPPHIRE SKIN & AESTHETICS

01. HOW THIS STARTED

We tested twenty clinics. Yours was one of them.

Over the past few weeks we've researched 31 elective clinics across Bengaluru — skin, aesthetics, hair and dental. Twenty of them we tested end to end: a real enquiry, sent the way a patient would send it, timestamped, then checked again later to see what became of it.

No survey. No scraped list. We sent the message and waited.

WHAT WE CHECKED

Response time on the channels you publish. Whether anything was asked before a price was quoted. Whether a second follow-up ever came. Public review patterns and live ad activity.

WHAT WE COULDN'T CHECK

Anything inside your systems — how many enquiries you actually receive, and whether the ones that went quiet booked anyway. Those need your data. We've marked them unknown rather than guessed.

02. YOUR CLINIC

What we saw from the outside

WHAT'S CLEARLY WORKING

Two sentences on what the clinic is verifiably doing well : review volume and rating, credentials, treatment range, or the response quality where it was good. Specific and factual, not flattery.

WHAT WE OBSERVED

A patient enquiry sent to Sapphire's WhatsApp Business number on a Sunday, after hours. No automated greeting and no qualification sequence was triggered.

WHERE IT STOOD LATER

Still unanswered at an 11:00 AM follow-up check the next morning. A separate enquiry sent through Instagram DM received no automated response either.

WHAT WE COULD SEE FROM OUTSIDE

Around 7 active Google Ads campaigns — acne scar treatment, hair and nail disorders, general dermatology — running to two Bengaluru locations, JP Nagar and Koramangala. Website carries booking CTAs and WhatsApp contact, with no auto-reply or chatbot on either channel.

One test can't tell us whether this repeats, or whether that patient booked anyway. What it does tell us is that right now there's no easy way for anyone at SAPPHIRE SKIN & AESTHETICS to know either and that's the part worth fixing, regardless of what we found.

03. WHAT REPEATS

Five things we saw again and again

Conversion, not attention

In 18 of the 20 clinics we tested, the problem wasn't getting noticed by patients. It was what happened after someone raised their hand.

Automation stops at hello

The clinics already running automated replies used them to acknowledge, not to qualify. Nothing asked, nothing sorted.

After-hours enquiries wait

Nearly every test sent between 9 PM and 1 AM sat untouched until the next morning, often 9 to 11 AM.

One person holds it together

In founder-led clinics, enquiry handling quietly depends on the one or two people who are also the reason patients came.

Spend doesn't predict follow-up

Clinics running 15 to 44 active campaigns showed the same gap as clinics running none.

04. THE COST

The lost patient is the part you can see

FIRST ORDER

Enquiries contacted within five minutes are 21× more likely to qualify than those contacted after thirty.

— MIT / Oldroyd Lead Response Study

Lead-to-consultation conversion falls from 35–55% to under 20% once an enquiry sits overnight.

— Ichelon Consulting, India

India OPD no-show rates run 15–30%. An empty chair still costs the room, the hour, and the spend that acquired it.

— DocTrue, India

A Bengaluru clinic with 300 WhatsApp enquiries a month converted under 30% at a 4.5-hour response time. After fixing response and follow-up: over 70%, same volume.

SECOND ORDER

Every hour logging is an hour not with a patient.

Copying details between WhatsApp, the diary and the tracker is time not spent on a call, or reassuring a nervous first-timer.

Your first impression isn't clinical.

By the time a patient meets you they've formed a view. Your doctor hasn't spoken to them. Your front desk has.

Revenue sits in one person's memory.

Coordinator tenure in India typically runs under a year. When they leave, the follow-up list leaves with them.

Growth makes it worse before better.

A recurring pattern: campaigns start working, volume climbs, and the owner asks the agency to slow down.

05. THE EXCEPTION

One clinic in twenty cleared the bar

Of the twenty clinics we tested, one replied in under a minute - and asked a question back before quoting anything.

Under a minute, and one question asked before a price. That is the entire standard — and nineteen of twenty missed it, including the clinics already running automation.

It was done by hand, which is exactly why it isn't repeatable. One person can reply in a minute once. Nobody can do it at 9 PM on a Sunday, mid-consult, with a walk-in waiting at the desk.

Handle it with a system and your team gets those hours back for the work that needs them — the call, the reassurance, the person in front of them. Which is also what turns into a review, and a referral.

06. WHAT BECOMES POSSIBLE

The work that shouldn't need a person

Nothing waits until morning.

Every enquiry gets a real acknowledgement within a minute — Sunday night included.

Nobody who showed interest disappears.

Second and third follow-ups go out whether or not anyone remembered.

Your team joins a conversation already started.

What they want, when they're free, whether they've been before — asked before anyone opens the chat.

Every enquiry has a record.

You can answer how many came in and what happened to them, without asking anyone to reconstruct it.

WHERE THIS STOPS — AND HOW YOUR PATIENT DATA IS HANDLED

System handles logistics: acknowledgement, scheduling, reminders, follow-up. It does not give medical advice, does not diagnose, and does not speak for the doctor. Every clinical conversation stays entirely yours.

Health data is sensitive personal data under the DPDP Act and the obligation sits with you. We build to that: consent captured before processing and revocable, purpose limited, marketing consent separate from treatment consent, data resident in India. You remain the data fiduciary; we operate as your processor.

07. BEFORE AND AFTER

What actually changes at the desk

TODAY

- Forty unread messages before the clinic opens
- The same five answers typed out again by hand
- Details copied from WhatsApp to the diary to the tracker
- “Let me check and get back to you” - and sometimes nobody does
- Follow-ups happen for the patients someone remembers

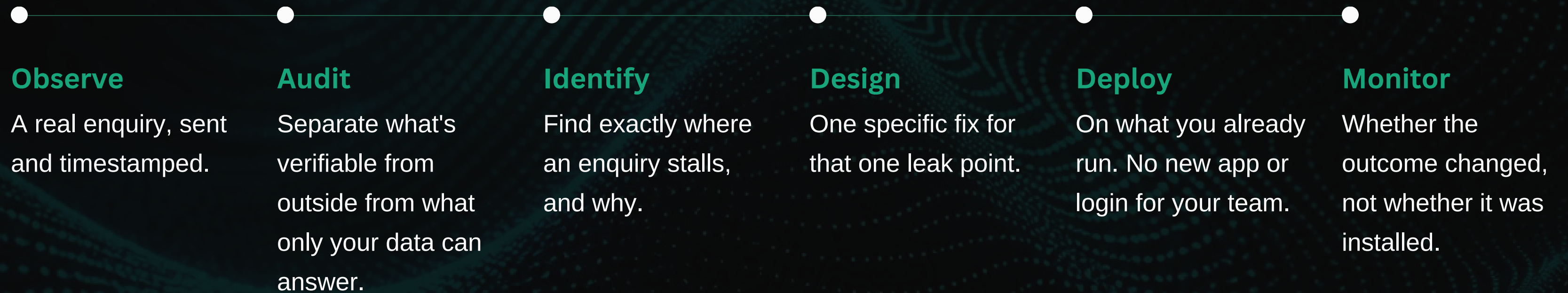
AFTER

- Arrives to a sorted list, not a backlog
- Answers only what genuinely needs a person
- Details land where they need to be, once
- Spends the day on the patients in the building
- Follow-ups run whether or not anyone remembered

The work doesn't disappear. It moves — off the keyboard, and back onto the people who came in.

08. THE METHOD

How we work



The free audit is the first three steps, done properly, for **SAPPHIRE SKIN & AESTHETICS**. Everything after that only happens if you want it to.

09. THE OFFER

A complete operational audit, at no cost

WHAT WE LOOK AT

- Response time across every channel a patient can reach you on
- Whether follow-up is consistent, and where it stops
- How patient communication holds up on a busy day
- Where enquiries are leaking, with the specific point named
- What a fix would look like, and whether one is worth doing

We'll be working with a small number of clinics right now - there are four of us, and each audit is done by hand rather than generated.

WHAT YOU GET

A written strategy document, specific to your clinic.

Not a call summary. A document you keep.

Yours regardless of what you decide.

Take it to your existing team, to another vendor, or act on it yourself. No clause, nothing to sign.

We'll tell you if there's nothing to fix.

If the audit finds you're already running clean, we'll say so.

We won't promise you a number of patients. Anyone who does is telling you something about themselves.

10. WHO WE ARE

The Minds Behind Valence Ops.



Tilak

Co-Founder & CEO



Maddy

Co-Founder & CTO



Pratham

Co-Founder & COO



Adithya

Co-Founder & CAIO

A 30 min audit, whenever suits — call, or in person.